

ATTACHMENT 1

REVISED PACKAGE INSERT

VENTAMOL TABLET 4 mg

VIVENXX-PC3

DESCRIPTION

8mm diameter. Round, light purplish uncoated tablet, bevel-edged, flat faces, "HD" embossed and scored on the same face.

COMPOSITION

Each tablet contains Salbutamol sulphate equivalent to Salbutamol 4mg.

ACTIONS AND PHARMACOLOGY

Salbutamol is a selective β_2 adrenoceptor agonist. At therapeutic doses it acts on the β_2 adrenoceptors of bronchial muscle, with little or no action on the β_1 adrenoceptors of cardiac muscle. It is suitable for the management and prevention of attack in asthma.

PHARMACOKINETICS

Salbutamol administered intravenously has a half-life of 4 to 6 hours and is cleared partly renally and partly by metabolism to the inactive 4'-O-sulphate (phenolic sulphate) which is also excreted primarily in the urine. The faeces are a minor route of excretion. The majority of a dose of salbutamol given intravenously, orally or by inhalation is excreted within 72 hours. Salbutamol is bound to plasma proteins to the extent of 10%.

After oral administration, salbutamol is absorbed from the gastrointestinal tract and undergoes considerable first-pass metabolism to the phenolic sulphate. Both unchanged drug and conjugate are excreted primarily in the urine. The bioavailability of orally administered salbutamol is about 50%.

INDICATIONS

Relief of bronchial asthma of all types, chronic bronchitis and emphysema.

CONTRAINDICATIONS

- Salbutamol should not be used for threatened abortion during the first or second trimester of pregnancy.
- Salbutamol and beta-blocking drugs such as propranolol should not usually be prescribed together.
- Salbutamol tablets are contraindicated in patients with a history of hypersensitivity to any of their components.

WARNINGS AND PRECAUTIONS

The management of asthma should normally follow a stepwise programme, and patient response should be monitored clinically and by lung function tests.

Increasing use of short-acting inhaled β_2 agonists to control symptoms indicates deterioration of asthma control. Under these conditions, the patient's therapy plan should be reassessed. Sudden and progressive deterioration in asthma control is potentially life-threatening and consideration should be given to starting or increasing corticosteroid therapy. In patients considered at risk, daily peak flow monitoring may be instituted.

Patients should be warned that if either the usual relief is diminished or the usual duration of action reduced, they should not increase the dose or its frequency of administration, but should seek medical advice.

Salbutamol should be administered cautiously to patients with thyrotoxicosis.

Potentially serious hypokalaemia may result from β_2 agonist therapy mainly from parenteral and nebulised administration. Particular caution is advised in acute severe asthma as this effect may be potentiated by concomitant treatment with xanthine derivatives, steroids, diuretics and by hypoxia. It is recommended that serum potassium levels are monitored in such situations.

In common with other β -adrenoceptor agonists, salbutamol can induce reversible metabolic changes, for example increased blood sugar levels. The diabetic patient may be unable to compensate for this and the development of ketacidosis has been reported. Concurrent administration of corticosteroids can exaggerate this effect.

As maternal pulmonary oedema and myocardial ischaemia have been reported during or following premature labour in patients receiving beta2-agonists, careful attention should be given to fluid balance and cardio-respiratory function, including ECG monitoring. If signs of pulmonary oedema and myocardial ischaemia develop, discontinuation of treatment should be considered.

Due to the risk of pulmonary oedema and myocardial ischaemia that has been observed during the use of beta2-agonists in the treatment of premature labour, before these products are given to any patient with known heart disease, an adequate assessment of the patient's cardiovascular status should be made by a physician experienced in cardiology.

Tocolysis: Serious adverse reactions including death have been reported after administration of terbutaline/ salbutamol to women in labor. In the mother, these include increased heart rate, transient hyperglycaemia, hypokalaemia, cardiac arrhythmias, pulmonary oedema and myocardial ischaemia. Increased fetal heart rate and neonatal hypoglycaemia may occur as a result of maternal administration.

Bronchodilators should not be the only or main treatment in patients with severe or unstable asthma. Severe asthma requires regular medical assessment including lung function testing as patients are at risk of severe attacks and even death. Physicians should consider using oral corticosteroid therapy and/or the maximum recommended dose of inhaled corticosteroid in those patients.

PREGNANCY AND LACTATION

Administration of drugs during pregnancy should only be considered if the expected benefit to the mother is greater than any possible risk to the foetus. As with the majority of drugs, there is little published evidence of its safety in the early stages of human pregnancy, but in animal studies there was evidence of some harmful effects on the foetus at very high dose levels.

As salbutamol is probably secreted in breast milk, its use in nursing mothers is not recommended unless the expected benefits outweigh any potential risk. It is not known whether salbutamol in breast milk has a harmful effect on the neonate.

DRUG INTERACTIONS

The effects of salbutamol may be altered by guanethidine, reserpine, methyl dopa, tricyclic antidepressants and monoamine oxidase inhibitors.

There is an increased risk of hypokalaemia if high doses of theophylline or high doses of corticosteroids are given with higher doses of salbutamol.

Halogenated anaesthetics

Owing to the additional antihypertensive effect, there is increased uterine inertia with risk of haemorrhage; in addition, serious ventricular rhythm disorders due to increased cardiac reactivity, have been reported on interaction with halogenated anaesthetics. Treatment should be discontinued, whenever possible, at least 6 hours before any scheduled anaesthesia with halogenated anaesthetics.

Anti-diabetics

The administration of beta-agonists is associated with a rise of blood glucose, which can be interpreted as an attenuation of anti-diabetic therapy; therefore individual anti-diabetic therapy may need to be adjusted.

Potassium depleting agents

Owing to the hypokalaemic effect of beta-agonists, concurrent administration of serum potassium depleting agents known to exacerbate the risk of hypokalaemia, such as diuretics, digoxin, methyl xanthines and corticosteroids, should be administered cautiously after careful evaluation of the benefits and risks with special regard to the increased risk of cardiac arrhythmias arising as a result of hypokalaemia.

MAIN SIDE/ADVERSE EFFECTS

- **Immune system disorders:** Hypersensitivity reactions including angioedema, urticaria, bronchospasm, hypotension and collapse.

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- **Metabolism and nutrition disorders:** Hypokalaemia.
- **Nervous system disorders:** Tremor, headache, hyperactivity
- **Cardiac disorders:** Tachycardia, palpitations, cardiac arrhythmias including atrial fibrillation, supraventricular tachycardia and extrasystoles.
- **Vascular disorders:** Peripheral vasodilatation.
- **Musculoskeletal and connective tissue disorders:** Muscle cramps, feeling of muscle tension.

OVERDOSE AND TREATMENT

The preferred antidote for overdosage with salbutamol is a cardioselective beta blocking agent, but beta blocking drugs should be used with caution in patients with a history of bronchospasm.

Hypokalaemia may occur following overdose with salbutamol. Serum potassium levels should be monitored.

DOSAGE AND ADMINISTRATION

For oral use.

Salbutamol has duration of action of 4 to 6 hours in most patients. Increasing use of β_2 agonists may be a sign of worsening asthma. Under these conditions a reassessment of the patient's therapy plan may be required and concomitant glucocorticosteroid therapy should be considered. As there may be adverse effects associated with excessive dosing, the dosage or frequency of administration should only be increased on medical advice.

- **Adults:** The usual effective dose is 4mg three or four times per day. If adequate bronchodilatation is not obtained, each single dose may be gradually increased to as much as 8mg. However, it has been established that some patients obtain adequate relief with 2mg three or four times daily.
 - **Children:** The following doses should be administered three or four times daily:
 - 2 - 6 years : 1 to 2 mg
 - 6 - 12 years : 2 mg
 - Over 12 years : 2 - 4 mg
- Salbutamol Syrup is suitable oral therapy for children or those adults who prefer liquid medicines.
- **Special patient group:** In elderly patients or in those known to be unusually sensitive to β -adrenergic stimulant drugs, it is advisable to initiate treatment with 2mg three or four times per day.

Note: The information given here is limited. For further information consult your doctor or pharmacist.

Storage: Store below 30°C. Protect from light and moisture.

Presentation/ Packing: Blister of 10 x 10's

Manufactured by: HOVID Bhd.

121, Jalan Tunku Abdul Rahman, 30010 Ipoh, Malaysia.

Information date: September 2016

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